SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Nivolumab, paclitaxel and carboplatin before surgery

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Neoadjuvant NSCLC · selected paclitaxel / carboplatin backbone · NCCP 00849 · version 2 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment is given before planned surgery for resectable non-small-cell lung cancer at high risk of returning. It aims to treat the cancer before the operation and reduce the chance of recurrence. Surgery and benefit are not guaranteed.

Schedule
Up to three 21-day cycles. Day 1: nivolumab, paclitaxel and carboplatin. Then surgical MDT reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation in multiple organs
- ☐ Infusion reactions
- ☐ Delayed effects after treatment stops

Paclitaxel
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Peripheral neuropathy and hair loss
- ☐ Allergic reactions
- ☐ Muscle and joint pain

Carboplatin
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Nausea and vomiting
- ☐ Kidney, hearing and nerve effects
- ☐ Allergic reactions

Take the prescribed paclitaxel allergy-prevention medicines; your team reconciles steroids with anti-sickness treatment. Carboplatin dosing uses a verified kidney-function measurement; report dehydration or a change in health. Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose. Discuss severe or fatal immune inflammation and chemotherapy effects, including lasting hormone replacement, organ injury and effects on surgery timing.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00849 v2.

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